



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : نشوي محمود متولي الصديق

العنوان : ش الجيش

الحي / المدينة : كفر شكر

المحافظة : القليوبية

تاريخ تسجيل الإستمارة : 26-09-2026

رقم البطاقة : 27807161402225 رقم التليفون : 01091104546

العمر (تاريخ الميلاد) : 16-07-1978 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني الوظيفة: التوقيع:
الاسم: د/ندى حسن الوظيفة: التوقيع:
الاسم: د/نرمين رمضان الوظيفة: التوقيع:
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... /د. /..... /د. /..... /د. /..... /د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: نشوي محمود متولى الصديق National ID: 27807161402225	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

DMS
تدعيه
محمود + منى



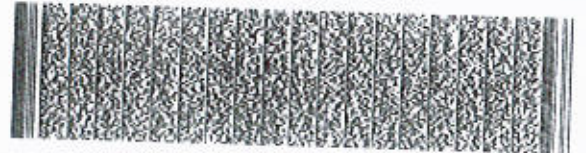
٢٠٢٥/٠٧ ٢٧٨٠٧١٦١٤٠٢٢٢٥

حاصله على دبلوم صفائح

أنثى متعلمة متزوجة
متولى اسماعيل متولى السمري
البطاقة سارية حتى ٢٠٢٢/٠٧/٠٩



١١



جمهورية مصر العربية
بطاقة تعريف شخصية

نشوى
محمود متولى الصديق السمري
٤ ثن الجيش - البر الشرقي
مركز كفر شكر - القليوبية

١٩٧٨/٠٧/١٦

٢٧٨٠٧١٦١٤٠٢٢٢٥

LF3515901



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



6-Patient ID:

1- Name : نشوى محمود متولى الصديق

2-ID number: 27807161402225

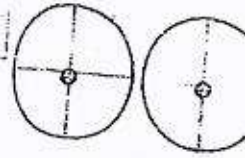
Sex:

MDT form
3- Medical number:
Female

83181206
Old Med :

Old Med :
58-2024

Patient Age : 46 Y



Family history

Family history

Phone

Phone

01091104546

BMI

BMI

Marital status

Marital Status

NRS Score

NRS Score

GROUP

GROUP

Co morbidity

Co morbidity

Menopausal
status

Menopausal Status:
PREMENOPAUSAL

Initial STAGE

Initial STAGE

REASON OF MDT

REASON OF
POST OPERA

Date of MDT

Date of MDT
11/02/

Breast Imaging :

Clinical data

Mets Workup :

BX

IDC

ER

8/8

ER COMMENT:

PR

4/8

PR COMMENT:

Her 2 Neu

2

Her 2 Neu COMMENT:

SISH

Amplified

SISH COMMENT :

KI67

40

N

M

0

Sonomammography

lt SM inner quadrant mass 16 x15 mm + lt axillary LN

PATIENT HISTORY

lt mrm 8/2023 ILC LN 7/21 + nodal necrosis +capsular relapse , free margin
received CTH 4 AC 10/2023 till 12/2023 at elkasr el-ainy

MDT OUTCOME & PLAN

Taxotere every 3 ws/ 4 CYCLES & Herceptin SC ANADJUVANT RTII . ADJ AI AND ZOLADEX

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

SURGERY

SURGERY

Psychiatric

Psychiatric

SIGNATURE

RADIOLOGY

PATHOLOGY

Nutrition

RADIOLOGY

PATHOLOGY

PATHOLOGY

Nutrition

Patient Name : الأستاذة / نشوى محمود متولى الصديق السمرى
 Patient No. : 27807161402225
 Path No. : SR-37/24
 Sex : Female
 Age : 45 Y
 Referring Doc. :
 Request No. : 13008223
 Request Date : 10/01/2024
 Verification Date : 22/01/2024
 Financial Category : اورام دار السلام - نفقة الدولة
 Department : Out Patient
 IHC No. :
 Registered by : marwa mohamed youssef

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast cancer
 Nature of Specimen: Slide revision.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

- * IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8).
- PR: Moderate nuclear reaction in 30% of tumor cells (Allred score 5/8).
- Her2: Moderate complete membrane reaction in >10% of tumor cells (score 2+).
- Ki67: 40% nuclear reaction of tumor cells.

Conclusion:

Left breast cancer, Slide revision:
 Invasive duct carcinoma, Grade III.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 5/8).
- Her2: Equivocal (score 2+). For SISH.
- Ki67: Positive in 40% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0,

Verified by :

Printing Date: 03/02/2024 09:42:14

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
 Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name الأستاذة / نشوى محمود متولى الصديق السمرى
Patient No. 27807161402225
Path No. SR-37/24
Sex Female
Age 45 Y
Referring Doc.

Request No. 13008223
Request Date 10/01/2024
Verification Date 22/01/2024
Financial Category اورام دار السلام - نفقة الدولة
Department Out Patient

Registered by .marwa mohamed youss

IMMUNOHISTOCHEMISTRY REPORT

no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr. Rana Fathy
Dr. Toka Medhat

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Page 2 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	: الأستاذة / نشوى محمود متولى الصديق السمرى	Request No.	: 13008223
Patient No.	: 27807161402225	Request Date	: 10/01/2024
Path No.	: SR-37/24	Verification Date	: 22/01/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 45 Y	Department	: Out Patient
Referring Doc :		Registered by	: marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer.
Nature of Specimen: Slide Revision.

Gross Examination:
- One paraffin block labeled by (2831102) and 12 slides were received.

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade III (3+3+2).
* DCIS: No.
* Skin involvement: Free skin
* Lymph nodes: Positive (5/12).

Diagnosis:
Left breast cancer , Slide revision:
Invasive duct carcinoma, Grade III.
Positive lymph nodes (5/12).
- For further HC (ER, PR, Her2, Ki67).

Signature
Dr. Rana Fathy
Dr. Toka Medhat

Verified by :

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Name	Mrs. Nashwa Mahmoud Metwally El-sedeek
Ref. By Prof. Dr.	Ahmed Hamed Rashad
ID	6623017478
Date	10/10/2023
Acc. No.	281041098

6623017478

¹⁸F-FDG PET/CT Scan

Indication of the study: Status evaluation study for a case with previous history of managed left breast malignancy ILC pathology 8.2023.

Total body PET scan was done 40 minutes after IV administration of 300 MBq of ¹⁸F-FDG.

CT scanning was performed simultaneously without IV contrast material injection.

Semi-quantitative analysis would be done for selected ROIs & the normal threshold would be the normal blood pool liver SUV....< 2

Reported By:

Dr. Islam Aboulwafa, MD

Approved By:

Prof. Dr.

Essam El-Shiekh

Head & Neck:

- Normal physiological biodistribution over scanned portion of the brain. (N.B; not all brain metastatic lesions could be detected on 18-FDG PET/CT due to physiologic uptake in brain. MRI brain is suggested in cases of clinical suspicion).
- Normal naso-pharynx, larynx, thyroid (Non-homogenous thyroid gland, for further thyroid work-up) and salivary glands.
- No enlarged FDG hypermetabolic cervical L.Ns seen.

Torso:

Thorax:

- *Low activity operative bed reticulations.*
- *No focal operative bed active masses.*
- *No enlarged FDG hypermetabolic mammary related lymph nodes.*
- *For mammographic correlation.*
- Bilateral tracer starved atelectasis and sub-pleural reticulations.
- No FDG hypermetabolic pulmonary nodules or masses seen.
- No enlarged FDG hypermetabolic hilar or mediastinal L.Ns seen.
- No pleural or pericardial effusion seen.

Abdomen & Pelvis:

- Average size of the liver with no FDG hypermetabolic focal lesions. For dynamic MRI study of the liver assessment if warranted.
- No biliary tree dilatation.
- Cholecystectomy.
- Average size of the spleen with no focal active lesions.

Name	Mrs. Nashwa Mahmoud Metwally El-sedeek		
Ref. By Prof. Dr.	Ahmed Hamed Rashad	Date	10/10/2023
ID	6623017478	Acc. No.	281041098

- Normal appearance of pancreas (no sizable active masses), adrenals and pelvic organs (endometrial activity with no sizable masses likely physiologic, for US correlation).
- No enlarged FDG hypermetabolic abdominal or pelvic L.Ns seen.
- Congested mesentery with strandy reticulations.
- Normal physiological biodistribution over the bowel (non-uniform bowel uptake with no sizable masses) & urinary tracts.

Surveyed skeleton:

- Faint marrow uptake of SUVmax. 2.5 yet no tracer active lytic or sclerotic osseous lesions seen, likely hematopoietic, for CBC correlation and follow up.
- Scattered sclerotic non-metabolically active foci of the spine and pelvic bones (mostly degenerative changes).

Opinion:

- The operative bed changes are mostly of post management sequelae.
- The surveyed body is free from any FDG hypermetabolic neoplastic masses.
- For mammographic correlation.

Thanks for your referral;

Dr. Ahmed Kandeel
MD Nuclear Medicine

Dr. Islam Aboulwafa, MD
Prof. Dr. Essam Ali El-Shiekh



Handwritten signature of Dr. Islam Aboulwafa.



Patient Name: Nashwa Mahmoud Metwally Al Seddik Al Semary	Date : 23/03/2025
Exam : C.T CHEST WITH CONTRAST	Age : 46 Year(s)
Medical ID: 58/2024	Pat. ID: 98169

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

- Reported case of left MRM.

I Chest:

- Status post left MRM showing no operative bed gross masses or sizable collections.
- No sizable pulmonary nodules, masses, areas of ground glass attenuation, consolidation or cavitary lesions.
- No CT evidence of pathological mediastinal lymph node enlargement.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross enlargement.

II Abdomen & pelvis:

- Enlarged liver showing homogeneous parenchymal attenuation. No sizable focal lesions or biliary radicles dilatation.
- Right renal cortical lesions as follows:
 1. Lower polar medial cortical hetergeonously enhancing mass lesion with central linear lucency measuring 2.8 X 2 cm.
 2. Lower polar lateral mixed solid and cystic lesion measuring 1.4 X 1.2 cm.
 3. Lower polar fat containing lesion mostly angiomyolipoma measuring 15 mm
- Normal CT appearance of the spleen, pancreas, suprarenals glands, aorta, left kidney and IVC.



Patient Name: Nashwa Mahmoud Metwally Al Seddik Al Semary	Date : 23/03/2025
Exam : C.T CHEST WITH CONTRAST	Age : 46 Year(s)
Medical ID: 58/2024	Pat. ID: 98169

- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- No pathological abdominal or pelvic lymph node enlargement.
- No ascites.
- IUD is seen.
- Right iliac wing sclerotic area.
- Right femoral and left ischial sclerotic foci mostly bone islands.
- 1st degree lytic spondylolisthesis of L5

Opinion:

Status post left MRM showing no operative bed gross masses or sizable collections.

- No sizable pulmonary or hepatic deposits.
- Right renal two heterogenous lesions as described, lower polar Bosniak III (lateral smaller lesion) and Bosniak IV (medial larger lesion) lesions for further assessment.
- Another right renal lower polar fat containing lesion mostly angiomyolipoma.
- Right iliac wing sclerotic area of bone scan.

Dr. Mohamed Mohsen, M.D.



Pat. Name: نشوى محمود متولى السمرى	Pat. ID: 662730
	Date : 20/08/2023



PELVI-ABDOMINAL SONOGRAPHY REVEALED:

- *Liver: Average hepatic size with normal homogenous echopattern. No intrahepatic focal lesions or biliary radicles dilatation. Normal calibre of P.V & C.B.D.
- *Spleen: Normal size & pattern.
- *No ascites.
- *Gall Bladder: Not visualized (cholecystectomy).
- *Normal pancreas and retroperitoneal area.
- *No para-aortic or mesenteric lymphadenopathy.
- *Kidneys: Normal global dimensions of both kidneys with normal parenchymal thickness and echopattern. No renal calculi or back-pressure changes.
- *U.Bladder: Normal appearance with no bladder calculi or intravesical filling defects.
- *Normal pelvic sonography with no abnormal adnexal mass or cyst.

CONCLUSION:

Normal abdominal & pelvic U/S.

Prof. Dr/ AHMED FARID
Dr/ MOHAMED SALAH

NADA

01064035999

ق قليب
ناصر العوام

ق شبيب القناطر
بجوار السجل المدني

ق بنها
ميدان سعد زغلول